

courageous communication, and diving in with one partner's exiles, we continually monitor the needs of our own inner community.

In the third phase, we come to repair, forgiveness, reconciliation, and the future. By this time, couples are clear about their parts, their partner, and their relationship, and they are less reactive with each other. Their intimacy is deeper, and it's time for them to envision their future. If the therapist has not already initiated a conversation about apology and forgiveness, this is the time. We support partners in exploring obstacles to apology and forgiveness, facilitate authentic repair, and help them share responsibility for the health of their relationship. When couples have more safety and trust, they can design a future based on new ways of thinking, feeling, and being.

THE THREE PHASES OF IFIO COUPLE THERAPY

Phase 1: Learning

- Meet the couple to assess their level of differentiation.
- Learn what they fear.
- Learn their emotional needs, hopes, and intentions for therapy.
- Offer possibilities.

Phase 2: Getting to the Heart of the Matter

- Teach the couple new communication skills to help them unblend and regulate their ANS.
- Get permission to be their *parts detector*.
- Track how they fight and how they're vulnerable, including:
 - o How they negotiate needs
 - o How they shame each other
 - o How they receive shaming (i.e., how their protectors shame them)
- Walk the couple through the new communication behaviors you want them to learn.
- Get them in their bodies and deepen their work with experiential exercises.
- Promote and reinforce relational unburdening.
- Do individual work, including:

- o Unblending
- o Unburdening shamefulness
- o Having the partner as a witness
- o Practicing new communication skills
- o Returning to relational needs
- o Practicing skills that are more likely to get needs met

Phase 3: Ending

- Help the couple see each other as a resource rather than as the “wounder” or the “healer.”
- Heal betrayals by guiding them in the process of repair and forgiveness.
- Provide support and guide partners as they gradually unblend and (if they decide to move forward together) develop a relationship based on acknowledging differences as well as sharing a vision.

PHASE 1 OF TREATMENT: LEARNING

Assessment: IFIO is Collaborative and Non-Pathologizing

Assessments in psychotherapy are often structured around diagnosing pathology. In IFIO, as in IFS, we don't categorize clients by diagnosis. In our view, the human mind consists of an internal system of parts and an undamaged Self. That internal system, like human systems at all levels, organizes itself to protect vulnerability—and does so with brilliance and creativity. But ultimately, parts do not have the resources to navigate a dangerous world alone; they need the Self and we aim to introduce them.

We begin therapy with interest and curiosity, meeting and greeting the couple's parts, learning about the beliefs and priorities of each partner's system, and getting a look at the system they've created together. In many cases, the couple's protectors will be blaming each other, and they will be stuck in a painful, self-absorbed “I” state that feeds on disconnection. Our goal, which we convey early on, is to help them reclaim their “we” state. After connecting with both partners, we communicate a few important facts about our role and the stance of the IFIO therapist:

1. We are not afraid of their parts.
2. We prioritize the safety of both partners.

3. We don't choose sides in disagreements.
4. We are open and curious about each partner's perspective.
5. We hold both partners with respect and curiosity, recognizing the validity of their experience and the vulnerability that drives their protective parts to extremes.

Ask Initial Questions, Listen, and Observe

To get a feel for the state of any given couple, we ask questions like:

- Why are you here?
- How do you feel about being different from each other?
- What do you hope for in your relationship?
- What do you wish to achieve in therapy?
- What do you fear most?

As we listen, we have the opportunity to gauge how differentiated they are from each other; to notice how differentiated their parts seem to be internally; to appraise the roles and relationships of their parts, inside and out; to estimate their baseline level of ANS dysregulation; to track and reflect back what each says accurately to anchor their narrative in our mind and help them feel safe; and to validate their feelings, large and small.

We also ask questions about their histories. Have they had a problem with: (1) mental illness, (2) drug or alcohol abuse, (3) their family of origin, (4) children and parenting, (5) sexuality, (6) betrayal, (7) cultural differences, (8) prejudice and bias in their larger context, (9) past therapy, or (10) apologizing and forgiving? This inquiry, which could last several sessions, gives us information that may prove relevant as therapy unfolds.

Introducing the IFIO Model

Depending on a couple's needs, IFIO can be introduced by talking about it or by diving in experientially and showing how it works. Regardless of how we choose to introduce the IFIO concepts, we include the following messages:

- All human beings possess *parts* and an undamaged *Self*.
- No single feeling or experience represents all of who we are at any given moment; parts are aspects of a larger whole.

- Some parts take on protective roles to ward off vulnerability, other parts are vulnerable, and still other parts have not been affected by wounding.
- IFIO is collaborative: our contract with you depends on your wishes and desires, which may change during therapy.
- You are free to confront, ask questions, disagree, and express yourselves continually. Our motto is “all parts of you are welcome.” Your parts may need time to trust this invitation.
- We will start by understanding the parts who get caught in painful relational tangles.

AN INITIAL SESSION

Avi and Hannah were a cisgendered, heterosexual, European-American couple in their late thirties. They had been together, not married, for eight years total and had lived in New York City for three years. In addition to having many values in common, both were successful visual artists.

Their relationship had been very passionate until Hannah told Avi that she was ready to be married and have children. Avi responded that he was unsure about making a lifetime commitment. As their conversations about getting married became increasingly fraught and painful, they agreed they were at an impasse and sought therapy.

Therapist: Welcome. We spoke briefly about what’s going on, but I want to start today by hearing more from you both. I’d like to know what’s happening, what you wish for in therapy, and how I can help.

Inviting, listening, and observing

Avi: Clearly, I’m the problem. [*looking at Hannah*] I know you want to get married. We always said we would. And do the baby thing. Now I’m not sure.

Therapist: You’ve been together for eight years. It sounds as if you have made some assumptions about where your relationship was headed, but now you, Avi, aren’t sure.

Avi: [*nods*] That’s the short of it.

Therapist: And I hear you have a part who thinks you’re the problem.

Introducing parts language

Avi: Well, Hannah thinks that.

Hannah: Yes, I do actually. It's not that he's the problem per se, but something's up all of a sudden. One minute we're on the same page, and the next he's all squirrely about commitment. I don't get it. I'm upset, angry, and afraid.

Therapist: Can you say more about your fear?

Inviting vulnerability

Hannah: I'm 35. If I'm going to have babies, I need to start yesterday. I love Avi. We have a great life. I would be devastated if our relationship ended.

Therapist: You make sense to me. From what you're saying, Hannah, this change in Avi is sudden, confusing, and scary?

Hannah: Yes. I feel sick just talking about it.

Therapist: And when the two of you try to talk about it, what happens?

Hannah: What happens, Avi?

Avi: I'm not sure. I freak out. I get panicky and feel this urge to run like hell.

Therapist: *[looking at Hannah]* And you?

Hannah: I get incredibly frustrated. I'm afraid he's going to run away, and I try to hold him in conversation. But I'm panicking too.

Therapist: I understand. Both of you have parts who freak out and panic. But you manage panic very differently. Am I right?

Hannah: That's true.

Therapist: *[addressing both]* Take a minute with my next question. What is the panic trying to communicate?

Offering a gentle invitation to inquire and listen internally

Avi: *[after a moment]* I'm trapped.

Hannah: The opposite. I'm alone.

Therapist: This sounds very hard. I can see how and why you need help now.

Validating

Hannah: I'm at the place where I need some answers and decisions.

Avi: Okay, so remember I mentioned feeling panicky? Well, here it is, right here in my throat.

Therapist: Can I help?

Avi: This sounds like an ultimatum. Yes, help would be great.

Therapist: Take a couple of deep breaths. [pauses] As strange as this next idea might seem, notice your throat and say "hi" to that feeling. Do you hear yourself saying any words to yourself?

*Facilitating inner differentiation by
noticing the feeling instead of being the
feeling*

Avi: [pauses] No. This is a sensation.

Therapist: Okay. Sensations let you know that something important is happening. I can help you and Hannah unpack all this and figure it out. That may take some time, and if you do this, you will have a lot of feelings before we're finished. But we'll start by talking about the process of being in therapy. My guess is that no hard-and-fast decisions will be made today.

*Offering help and fashioning containment
for therapy*

Avi: Yeah, good. No decisions is good by me.

Therapist: Hannah, do you want to say anything?

Hannah: Wow! Just the thought of marrying me kicks off a panic attack? That hurts.

Avi: Hannah, it's not the thought of marrying you. It's the ultimatum that freaks me out.

Therapist: I'm getting the picture of what's happening with you both, and I know there is so much more to talk about. However, I have some information about the therapy

process that I think will help. And I want to save time for questions about how I work, or about me in general.

Avi: Have you seen this before?

Therapist: I have, Avi. Many times.

Avi: And what happens?

Therapist: Everyone is different. This therapy will offer you both the safety to get curious about yourselves, to listen to each other and speak with each other differently, and to better understand what's happening. My experience tells me that once you have the capacity to stick with hard conversations without panicking, you will learn a lot more about each other, and the decisions you have to make will be easier.

Hannah: That would be good for me. I'm really in the dark here.

Therapist: I wonder if we can put decision making on hold until you've had the chance to do some exploring?

Hannah: That's a challenge for me.

Therapist: Let's listen to the part who just said it'll be a challenge and find out why.

Hannah: You think it's not all of me?

Therapist: Are you open to trying a little experiment with me to find out? Then I'll explain more.

Hannah: Okay.

Therapist: Stay with that sense of challenge for a minute. Where do you notice it?

Hannah: I hear these words in my head: *I need to know right now so I can deal with it.*

Therapist: So the voice in your head wants you to be prepared?

Hannah: Yes.

Therapist: And then what, Hannah?

Hannah: Then I can either relax or freak out.

Therapist: It wants to know the future?

Hannah: Exactly! I've been feeling so out of control.

Therapist: I hear you. As I said, if you work with me, we'll get to that. Right now, I will let you know how I work and what I believe is possible. How does that sound? [*they nod*]

Avi, you mentioned on the phone that you know a little bit about IFS and have shared that with Hannah. I'm not going to give you a discourse on this method of couple therapy, which is based on IFS, but I think the basic ideas are worth understanding. In this model, we recognize that the minds of all human beings have a multitude of parts, or aspects of personality. That's normal. I, too, have parts. This explains why we can be feeling and thinking many different things at the same time. Are you with me so far? [*both nod*] What's nice about this for couples is realizing that one thought or feeling is never all of who you are. As you get to know your parts, you will understand yourself and each other better. For example, you have both been describing feeling defensive but also very vulnerable. We can find and help those different parts of you.

Hannah: Can you remind us what they are?

Therapist: I sure can. So you, Hannah, have described getting frustrated, a cousin of angry, right? To this way of thinking, you have a part who gets frustrated. You also described feeling afraid and uncertain. We would say that's a completely different part of you. One who feels much more vulnerable.

Hannah: Yes, they are different. One is strong and the other is weak.

Therapist: And you, Avi, said you get freaked out and scared. And then you want to run away. These are parts of you.

Avi: I understand what you're saying. This will take some time to get my head around, but I see what Hannah means. One part feels weak and one feels strong.

Therapist: The concept of parts can take time to grasp. It takes experience mostly. We could say one part feels protective and the other feels vulnerable. When these parts get caught up in your conversations, things get heated, and it's hard for you to slow down. Both of you are left feeling upset, is that right?

Emphasizing that some parts take on protective roles to ward off the vulnerability of other parts

Avi: And then we feel really disconnected.

Hannah: And unsafe!

Therapist: That’s right. Disconnected and unsafe. We don’t have much time left today. I want you to go home and think about what we’ve discussed. If you decide to explore all this with me, I will gather more information about your histories, your families, and your hopes for therapy on an ongoing basis. In the meantime, I want you both to hear that I’m tracking what a painful time this is. And I hear that you could use help.



This is just one example of how an initial IFIO session might go. Every couple and every therapist are different. We encourage you to continue using any assessment tools you value and to find your own way of introducing the IFIO concepts while staying open, curious, and connected to the couple.

PHASE 2 OF TREATMENT: GETTING TO THE HEART OF THE MATTER

Tracking the Sequences of the Couple’s Conflict

Salvador Minuchin, a family therapist in the late twentieth century, developed the strategy of *tracking* as a means of exploring negative interactions in families (Minuchin & Fishman, 1981). Similarly, IFIO therapists use tracking to assess a couple’s predictable, repetitive, negative interactions.

Although couples often believe that one of them has “started” a fight and is to blame, we use the technique of tracking to shine light on process and move the conversation off the content of any given disagreement. While the couple’s quarreling protectors want to discuss what happened this time and who’s to blame, the relevant question is what happens every time they get into conflict and why their protective parts keep doing this regardless of content. To call attention to the feelings and needs underneath painful interactions, we not only unpack the couple’s external, *interpersonal* dynamics, we also unpack their internal, *intrapersonal* dynamics, which motivate protectors. As long as protectors keep activating over the course of therapy, we track their interactions.

TRACKING

- Differentiates parts from the Self
- Helps partners notice protector reactivity in the form of predictable sequences of negative interactions
- Reveals the feelings and needs of vulnerable parts, which fuel protective behavior

- Invites partners to try something new so they can experience a different outcome

To track a sequence effectively, we stick with the following roadmap. First, we listen carefully to the couple's interaction and get details about each partner's behavior ("What do you do or say to your partner?"). Although couples believe the content of their fight is important, we stick with tracking how they interact. Next, we interview protectors about their motives ("What is this part afraid would happen to you if it didn't get so angry on your behalf?" or "What does this part hope for?") to learn about their hopes, fears, and needs. Rather than telling people how they feel or what motivates their protectors, we invite a process of self-exploration.

Our motto is "Ask—don't tell." By exploring, we flesh out the cycle of their fights experientially and help them recognize the predictability and repetitive nature of their fights.